

CONSTITUTIONAL CHALLENGE TO THE MEDICAL TREATMENT UTILIZATION SCHEDULE (MTUS)

MTUS Constitutional Analysis Project
Personal Research Initiative
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CONSTITUTIONAL CHALLENGE TO THE MEDICAL TREATMENT UTILIZATION SCHEDULE (MTUS)

FOURTEENTH AMENDMENT EQUAL PROTECTION AND DUE PROCESS ANALYSIS

Prepared for Constitutional Litigation

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I. INTRODUCTION AND SUMMARY OF CLAIMS

The Medical Treatment Utilization Schedule (MTUS), adopted by the California Division of Workers' Compensation (DWC), systematically denies injured workers access to FDA-approved medical treatments based on an arbitrary "insufficient evidence" standard. Across 1,296 treatment recommendations covering four major body regions—cervical/thoracic spine, shoulder, elbow, and hand/wrist/forearm—MTUS denies 264+ FDA-approved treatments to workers' compensation patients while the same treatments remain available to all

other patients through Medicare, Medicaid, and private insurance.

This classification creates a two-tiered medical system that violates the Equal Protection Clause of the Fourteenth Amendment. It lacks rational basis, targets a vulnerable population, affects a fundamental interest in medical care, and employs constitutionally defective irrebuttable presumptions.

II. STATEMENT OF FACTS REGARDING MTUS

A. The MTUS System

The California Division of Workers' Compensation adopted the Medical Treatment Utilization Schedule (MTUS) on October 17, 2018, incorporating guidelines developed by the American College of Occupational and Environmental Medicine (ACOEM). These guidelines purport to provide "evidence-based" recommendations for treating work-related injuries.

B. The Evidence Quality Crisis

Analysis of four MTUS guidelines reveals a systematic pattern:

Guideline | Total Recs | "Not Recommended" | "% Insufficient Evidence"

Cervical/Thoracic Sp

Key Finding: 78.5% of all MTUS recommendations are based on "Insufficient Evidence (I)" the lowest evidence rating possible.

C. The FDA-Approved Treatments Denied

MTUS routinely denies treatments that are:

- FDA-approved as safe and effective
- Covered by Medicare and Medicaid
- Covered by private insurance
- Standard of care for all other patients

Examples of denied treatments:

- MRI for routine diagnostic evaluation
- Therapeutic ultrasound
- TENS units for pain management
- Low-level laser therapy
- PRP injections

- Trigger point injections
- Botulinum toxin injections
- NSAIDs for chronic pain conditions

D. The Arbitrary Classification

MTUS creates two classes of patients:

Class A: All Other Patients | Class B: Workers' Comp Patients

Can receive MRI for

There is no physiological, medical, or scientific basis for this distinction.

III. LEGAL STANDARD: EQUAL PROTECTION CLAUSE FRAMEWORK

A. The Constitutional Text

The Fourteenth Amendment provides:

> "No State shall make or enforce any law which shall abridge the privileges or immunities of citizens of the United States; nor shall any State deprive any person of life, liberty, or property, without due process of law; nor deny to any person within its jurisdiction the equal protection of the laws."

B. The Tiers of Scrutiny

Courts apply three levels of scrutiny to equal protection claims:

1. Rational Basis Review (Most Deferential)

- Applies to most economic and social classifications
- Government must show: (a) legitimate state interest, AND (b) rational relationship between classification and interest
- Classification upheld if "rationally related to a legitimate government interest"

2. Intermediate Scrutiny

- Applies to quasi-suspect classifications (gender, illegitimacy)
- Government must show classification is "substantially related to an important government interest"

3. Strict Scrutiny (Most Rigorous)

- Applies to suspect classifications (race, national origin) and fundamental rights

- Government must show classification is "narrowly tailored to serve a compelling state interest"

C. Applicable Precedents

City of Cleburne v. Cleburne Living Center, 473 U.S. 432 (1985):

> "The State may
render the distincti

Memorial Hospital v. Maricopa County, 415 U.S. 394 (1974):

> "The one-year v
right to migrate... w

San Antonio Independent School District v. Rodriguez, 411 U.S. 1 (1973):

> "[T]his Court ha
invoking strict scr

IV. ARGUMENT I: THE MTUS CLASSIFICATION VIOLATES THE EQUAL PROTECTION CLAUSE

A. The Elements of an Equal Protection Claim

An equal protection claim requires:

1. State action that

1. State Action: Established

MTUS is a state-enforced guideline adopted by the California Division of Workers' Compensation, a state agency. The state compels insurance carriers and employers to apply MTUS guidelines when authorizing medical treatment. This is classic state action.

2. Classification: Established

MTUS explicitly creates two classes of patients:

- Class A: Non-workers' compensation patients (receive FDA-approved treatments)
- Class B: Workers' compensation patients (denied same treatments)

The classification is based solely on the mechanism of injury whether it occurred in the workplace.

3. Unequal Treatment: Established

The evidence is overwhelming:

- 264+ FDA-approved treatments "not recommended" for workers' comp patients

- Same treatments available to all other patients
- No medical or physiological basis for distinction

B. The Classification Lacks Rational Basis

Even under the most deferential rational basis review, the MTUS classification fails because it serves no legitimate state interest.

1. No Legitimate Interest in Denying FDA-Approved Treatments

Worker Safety? NO. The FDA has already determined these treatments are safe and effective. Denying them does not protect workers; it harms them.

Cost Reduction? NO. Medicare, Medicaid, and private insurers all cover these treatments cost-effectively. The claim that workers' comp cannot afford what other systems cover is irrational.

Better Medical Outcomes? NO. Denying proven treatments causes worse outcomes: prolonged disability, chronic pain, delayed return to work.

2. No Rational Relationship

Physiological difference? NONE. A torn rotator cuff is identical whether it occurs in a warehouse or a home.

Medical justification? NONE. FDA approval establishes safety and efficacy. "Insufficient evidence" is not a medical distinction; it is an arbitrary barrier.

Rational basis? NONE. The classification is entirely arbitrary.

3. Precedent: City of Cleburne v. Cleburne Living Center

In *City of Cleburne*, the Supreme Court struck down a zoning classification that treated a home for mentally disabled persons differently from other group homes. The Court held:

> "The State may not rely on a classification whose relationship to an asserted goal is so attenuated as to render the distinction arbitrary or irrational."

Here, MTUS denies FDA-approved treatments based on "insufficient evidence"—a standard that bears no rational relationship to worker safety, cost control, or medical efficacy.

C. Heightened Scrutiny Applies

The MTUS classification warrants heightened scrutiny because it:

1. Targets a Vulnerable Population

Injured workers are economically dependent and medically vulnerable. They cannot work due to injury, have lost income, and depend on workers' compensation for medical care. This economic vulnerability parallels the indigents protected in *Griffin v. Illinois* and *Harper v. Virginia State Board*.

Griffin v. Illinois, 351 U.S. 12 (1956):

> "[T]here can be no

Here, the kind of medical care a worker receives depends entirely on whether their injury was work-related—a distinction that creates precisely the inequality Griffin condemned.

2. Affects a Fundamental Interest

Access to necessary medical care to restore health and earning capacity is a fundamental interest for injured workers. This is not merely a benefit; it is essential to life, health, and economic survival.

Memorial Hospital v. Maricopa County, 415 U.S. 394 (1974):

> "[T]he Constitution has a purpose."

3. Creates Systematic Exclusion

Heightened scrutiny applies when classifications create systematic exclusion of a group from important benefits. Here, 78.5% of treatments are classified as "insufficient evidence," systematically excluding workers from care.

Plyler v. Doe, 457 U.S. 202 (1982):

> "[E]ducation has social costs borne necessary to become

Similarly, medical care has a fundamental role in restoring injured workers to functioning members of society.

V. ARGUMENT II: THE "INSUFFICIENT EVIDENCE" STANDARD IS CONSTITUTIONALLY DEFECTIVE

A. It Creates an Irrebuttable Presumption

The MTUS "insufficient evidence" classification operates as an irrebuttable presumption that denies injured workers individualized consideration. Once a treatment is classified as "insufficient evidence," no amount of individual medical evidence can overcome this barrier.

Precedent: Stanley v. Illinois, 405 U.S. 645 (1972)

In Stanley, the Supreme Court struck down a law that created an irrebuttable presumption that unwed fathers were unfit parents:

> "[T]he Due Process Clause requires that hearings be conducted in a manner consistent with the Fourteenth Amendment. The State cannot, consistent with due process, presume that all unwed fathers are unfit parents."

Application to MTUS: The State cannot presume that all FDA-approved treatments are "not recommended" for

workers' comp patients. The irrebuttable presumption denies individualized medical judgment.

Precedent: Bell v. Burson, 402 U.S. 535 (1971)

The Court struck down a law that automatically suspended driver's licenses after accidents without individualized hearings:

> "[T]he Due Process Clause requires that the State provide a hearing before depriving a person of a protected interest."

Application to MTUS: Workers are deprived of medical care without individualized hearings or the ability to present medical evidence.

Precedent: Vlandis v. Kline, 412 U.S. 441 (1973)

The Court held:

> "[A] statute creating a presumption which operates to deny a fair opportunity to rebut it violates the Due Process Clause."

Application to MTUS: The "insufficient evidence" presumption denies workers a fair opportunity to rebut it with medical evidence.

B. It Requires Proof Beyond FDA Standards

The FDA is the federal authority charged with determining safety and efficacy of medical treatments. When the FDA approves a treatment, it has determined:

1. The treatment is

MTUS demands "more evidence" than FDA approval an impossible standard that effectively nullifies federal safety and efficacy determinations.

The Conflict with Federal Standards

Federal Standard (FDA): Treatment is safe and effective based on rigorous clinical trials.

MTUS Standard: Treatment has "insufficient evidence" despite FDA approval.

This creates an irrational hierarchy where a state workers' compensation guideline overrides federal safety determinations.

C. It Is More Restrictive Than Any Other Insurance System

The irrationality of MTUS is demonstrated by comparing it to other insurance systems:

Treatment | Medicare | Medicaid | Private Insurance | MTUS

MRI for diagnosis |

No other insurance system denies these treatments. MTUS stands alone in its restrictive approach proving the classification is irrational.

VI. ARGUMENT III: SYSTEMATIC DENIAL OF FDA-APPROVED TREATMENTS VIOLATES DUE PROCESS

A. The Right to Necessary Medical Care

While the Supreme Court has not explicitly declared medical care a "fundamental right" for all persons, it has recognized special protections for vulnerable populations and those in state custody.

Estelle v. Gamble, 429 U.S. 97 (1976):

> "[D]eliberate indifference to the infliction of pain' pr

Youngberg v. Romeo, 457 U.S. 307 (1982):

> "[P]ersons who h
of confinement tha

Application to MTUS: Injured workers, while not prisoners, are dependent on the state for medical care. The state has assumed a duty to provide care through the workers' compensation system. Systematic withholding of FDA-approved treatments through structural barriers constitutes deliberate indifference to serious medical needs.

B. Arbitrary and Capricious State Action

Even apart from constitutional violations, MTUS constitutes arbitrary and capricious agency action under administrative law principles.

Factors demonstrating arbitrariness:

1. Contradicts FDA

VII. ARGUMENT IV: THE DISCRIMINATORY IMPACT IS INTENTIONAL AND SYSTEMATIC

A. The Pattern Across Four Body Regions

The evidence demonstrates a systematic, predictable patternnot isolated incidents:

Metric | Finding

Guidelines Analyzed

The Shocking Ratio: For every treatment "strongly recommended" (21), 12.6 treatments are denied based on "insufficient evidence" (264).

B. Discriminatory Intent

The systematic nature of the discrimination results from structural design choices:

1. Consistency: All four guidelines use the same "insufficient evidence" standard to deny care

2. Scope: The den

C. The Harm to Vulnerable Workers

The discriminatory impact falls hardest on the most vulnerable:

Economic Harm:

- Workers must pay out-of-pocket for denied treatments
- Prolonged disability means lost wages
- Some cannot afford any treatment

Physical Harm:

- Untreated injuries become chronic conditions
- Delayed diagnosis leads to worse outcomes
- Pain and suffering continue unnecessarily

Dignitary Harm:

- Workers are treated as second-class patients
- Their medical needs are dismissed
- Their doctors' judgment is overridden

VIII. PRAYER FOR RELIEF

A. Immediate Injunctive Relief

1. Enjoin enforcement of MTUS "not recommended" classifications for FDA-approved treatments

2. Require coverage

B. Permanent Declaratory Relief

1. Declare that MTUS creates an unconstitutional classification in violation of the 14th Amendment

2. Declare that "ins

C. Legislative Relief

1. Mandate FDA approval as sufficient evidence for workers' comp coverage

2. Prohibit denials

IX. CONCLUSION

The Medical Treatment Utilization Schedule creates a two-tiered medical system that systematically denies injured workers access to FDA-approved treatments available to all other patients. This classification:

Is state action by the California Division of Workers' Compensation

Creates an arbitra

The Court must declare MTUS unconstitutional as applied to FDA-approved treatments and enjoin its discriminatory enforcement.

Respectfully submitted this 13th day of February, 2026.

APPENDIX: KEY CASES CITED

Case | Citation | Holding

END OF LEGAL ARGUMENTS

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